

## Patient Details

Name : DUMMY REPORT  
UR :  
Ref :  
  
DOB : 06/12/12 Sex : Female  
IC NO. : Age : 12 Years  
Collected : 10/03/25 14:00 Ward :  
Referred : 10/03/25 Yr Ref. :

## Doctor Details

GRIBBLES IT DEPARTMENTAAAAA  
14 JALAN 19/1 2ND FLR  
WISMA KT (IF YOU RCV REPT  
PLS GIVE IT TO IT DEPT.)  
PETALING JAYA SEL 46300  
Lab No. : 25-9900033-I  
Courier Run : **WKT**  
Report Printed : 11/03/25 14:19

| ANALYTES                                 | RESULTS | UNITS                | REF. RANGES |
|------------------------------------------|---------|----------------------|-------------|
| <b>GENERAL SCREENING DBTICRP PROFILE</b> |         |                      |             |
| <b>HAEMATOLOGY</b>                       |         |                      |             |
| <b>Platelets</b> 血小板                     | 421     | x 10 <sup>9</sup> /L | (170-450)   |
| <b>BIOCHEMISTRY</b>                      |         |                      |             |
| Sodium 钠                                 | 139     | mmol/L               | (138-145)   |
| Potassium 钾                              | 4.1     | mmol/L               | (3.5-5.1)   |
| Chloride 氯化物                             | 99      | mmol/L               | (95-110)    |
| Urea 尿素                                  | 4.3     | mmol/L               | (2.5-7.5)   |
| Creatinine 肌酸酐                           | 78      | umol/L               | (30-90)     |
| Uric Acid 尿酸                             | 0.25    | mmol/L               | (0.13-0.39) |
| <b>SPECIAL LIPIDS</b>                    |         |                      |             |
| Apolipoprotein A 载脂蛋白 A                  | 1.25    | g/L                  | (1.08-2.25) |
| Apolipoprotein B 载脂蛋白 B                  | 1.02    | g/L                  | (0.60-1.41) |
| Apolipoprotein B/A ratio 载脂蛋白 B/A        | 0.82    |                      | (< 0.80)    |
| Lipoprotein(a) 脂蛋白 (a)                   | 89.90   | nmol/L               | (< 120.00)  |

NOTE: Effective 12/02/2020, we have subscribed to the EAS (European Artherosclerosis Society) recommended cut off [Lp(a)] For patients with intermediate or high CVD/CHD risk.

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| <b>SERUM/PLASMA GLUCOSE</b> |         |        |             |
| Glucose 葡萄糖                 | 5.5     | mmol/L | (3.9 - 6.0) |
| Specimen collected          | 08:45 h |        |             |
| Specimen type               | Fasting |        |             |

## Interpretation:

|                   |                                 |
|-------------------|---------------------------------|
| Category          | Fasting Plasma Glucose (mmol/L) |
| Normal            | 3.9 - 6.0                       |
| IFG (Prediabetes) | 6.1 - 6.9                       |
| DM                | >= 7.0                          |

IFG = Impaired Fasting Glucose; DM = Diabetes Mellitus  
Recommend Oral Glucose Tolerance Test (OGTT) for fasting plasma glucose levels 6.1 - 6.9 mmol/L.

Source: 2020 Clinical Practice Guidelines for the Management of Type 2 Diabetes Mellitus (6th Edition). Kuala Lumpur: Joint Publication of the Ministry of Health Malaysia, Academy of Medicine Malaysia, Malaysian Endocrine & Metabolic Society and Diabetes Malaysia.

NOTE: This sample was analysed from a non-preserved specimen.  
The glucose result MAY be falsely decreased.

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## SPECIAL CHEMISTRY

## SPECIMEN: WHOLE BLOOD

HbA1c 葡萄糖血红蛋白 5.1 % 32 mmol/mol

## Diagnostic Values of HbA1c in Malaysian Adults

|                |                       |                           |
|----------------|-----------------------|---------------------------|
| HbA1c (NGSP) % | HbA1c (IFCC) mmol/mol |                           |
| < 5.7          | < 39                  | Normal                    |
| 5.7 - 6.2      | 39 - 44               | *Prediabetes (IFG or IGT) |
| >= 6.3         | >= 45                 | T2DM                      |

IFG: Impaired Fasting Glucose; IGT: Impaired Glucose Tolerance; OGTT: Oral Glucose Tolerance Test; T2DM: Type 2 Diabetes Mellitus  
\*Recommend OGTT for HbA1c levels 5.7 - 6.2%

## Individualised HbA1c Target for Known Diabetes

|                |                       |                                                                                                      |
|----------------|-----------------------|------------------------------------------------------------------------------------------------------|
| HbA1c (NGSP) % | HbA1c (IFCC) mmol/mol |                                                                                                      |
| <= 6.5         | <=48                  | A: Tight target range for young, newly diagnosed diabetes without hypoglycaemia.                     |
| 6.6 - 7.0      | 49 - 53               | B: Target range for all other individuals not in category A or C.                                    |
| 7.1 - 8.0      | 54 - 64               | C: Target range for diabetes with comorbidities short life expectancy and/or prone to hypoglycaemia. |

Source: 2020 Clinical Practice Guidelines for the Management of Type 2 Diabetes Mellitus (6th Edition). Kuala Lumpur: Joint Publication of of the Ministry of Health Malaysia, Academy of Medicine Malaysia, Malaysian Endocrine & Metabolic Society and Diabetes Malaysia.

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|----------|---------|-------|-------------|

## LIVER FUNCTION

|                       |            |            |        |             |
|-----------------------|------------|------------|--------|-------------|
| Alpha 2 Macroglobulin | 阿尔法 2 巨球蛋白 | 2.65       | g/L    | (2.60-6.00) |
| Haptoglobin           | 触珠蛋白       | 0.3        | g/L    | (0.3-2.0)   |
| Bilirubin (Total)     | 总红胆素       | 11         | umol/L | (< 21)      |
| ** GGT                | 谷氨酰转肽酶     | <b>455</b> | U/L    | (< 51)      |
| ALT (with PLP)        |            | 16         | IU/L   | (< 43)      |
| AST (with PLP)        |            | 29         | IU/L   | (< 40)      |

## LIPID FUNCTION

|                   |         |      |        |             |
|-------------------|---------|------|--------|-------------|
| Apolipoprotein A1 | 载脂蛋白 A1 | 2.20 | g/L    | (1.08-2.25) |
| Total Cholesterol | 总胆固醇    | 3.9  | mmol/L | (< 5.2)     |
| Triglyceride      | 三酸甘油酯   | 1.23 | mmol/L | (< 1.70)    |

## LIVER FIBROSIS ASSESSMENT

|            |      |
|------------|------|
| FibroTest  | 3.10 |
| ActiTest   | 3.30 |
| SteatoTest | 3.20 |

**Body Mass Index** 身高体重指数 27.5 kg/sqm

Recommended BMI cut-off points for body weight classification and public health action for Malaysia

| Body Weight classification | BMI cut-off points for definition* (kg/sqm) | Co-morbidities risk                                 | BMI cut-off points for public health actions** (kg/sqm) |
|----------------------------|---------------------------------------------|-----------------------------------------------------|---------------------------------------------------------|
| Underweight                | <18.5                                       | Low (but increased risk of other clinical problems) | <18.5                                                   |
| Normal range               | 18.5 to 24.9                                | Low                                                 | 18.5 to 22.9                                            |
| Overweight                 | >=25.0                                      |                                                     | 23.0 to 27.4                                            |
| Pre-obese                  | 25.0 to 29.9                                | Moderate                                            | 27.5 to 32.4                                            |
| Obese class I              | 30.0 to 34.9                                | High                                                | 32.5 to 37.4                                            |
| Obese class II             | 35.0 to 39.9                                | Very High                                           | >= 37.5                                                 |
| Obese class III            | >=40.0                                      |                                                     |                                                         |

Source: \*WHO (1998) \*\* WHO Expert Consultation (2004)

Patient Weight 67 kg  
Patient Height 156 cm

NOTE: Original Fibronostic Score report copy to follow

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|------------------------------------|------------------------|-------------|--------------|
| <u>SPECIAL CHEMISTRY</u>           | <u>SPECIMEN: URINE</u> |             |              |
| RANDOM URINE ALBUMIN               |                        |             |              |
| ALBUMIN AND CREATININE RATIO (ACR) |                        |             |              |
| Urine Albumin                      | 13.1                   | mg/L        | (< 20.1)     |
| Urine Creatinine                   | 15.40                  | mmol/L      | (4.00-25.00) |
| ACR                                | 0.9                    | mg Alb/mmol | (< 3.5)      |

## KDIGO 2012 Albuminuria Categories:

- A1 Normal to Mildly Increased: <3
- A2 Moderately Increased: 3 - 30
- A3 Severely Increased: >30

Due to the variability in urinary albumin excretion, at least 2 specimens, preferably first morning void, collected within a 3 to 6 month period should be abnormal before considering a patient to have crossed one of these diagnostic thresholds. Exercise within 24 hours, infection, fever, congestive heart failure, marked hyperglycaemia, pregnancy, marked hypertension, urinary tract infection and haematuria may increase urinary albumin over baseline values.

**KFRE risk calculation is not applicable if eGFR is >=60.**

## ENDOCRINOLOGY

## SPECIMEN: SERUM

## HORMONE STUDIES

|                         |     |            |        |            |
|-------------------------|-----|------------|--------|------------|
| Serum Insulin           | 胰岛素 | 19.8       | uIU/mL | (2.6-24.9) |
| Fasting Plasma Glucose  |     | 6.7        | mmol/L | (4.1 -6.1) |
| Time                    |     | 09:00      |        |            |
| Homa IR                 |     | <b>5.9</b> |        | (< 2.6)    |
| Homa Beta Cell Function |     | 123.8      |        |            |

Please note that the reference ranges quoted are for a fasting sample.

Note:Effective 17/08/17,change of Insulin lower limit of reporting.

- NOTE: 1. Non diabetic cut off is <6.1 mmol/L based on the Malaysian Clinical Practice Guideline (CPG).  
2. It is advised that hypoglycaemia be determined based on the Whipple's triad.

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|--------------------------|------------------------|-------|-------------|
| <b>SPECIAL CHEMISTRY</b> | <b>SPECIMEN: SERUM</b> |       |             |
| NT-ProBNP                | < 5                    | pg/mL |             |

Note: Effective 23/11/2020, we are reporting the NT-ProBNP

Note: NT-proBNP values below 300 pg/mL have a 99% negative predictive value for excluding acute congestive heart failure (CHF). However, NT-proBNP levels can be elevated by a number of other conditions including acute lung injury, acute myocardial infarction, atrial fibrillation, cardiac amyloidosis, chronic obstructive pulmonary disease, chronic renal failure, cirrhosis, essential hypertension, hypervolemic states, left ventricular hypertrophy, pulmonary hypertension, pulmonary embolism with associated right ventricular dysfunction, and subarachnoid hemorrhage. Thus, it is important to interpret elevated NT-proBNP levels in light of a patient's other clinical conditions.

### Age-stratified cut-offs for diagnosis of acute heart failure:

| Age         | Cut-off      | INTERPRETATION                                                                                                                          |
|-------------|--------------|-----------------------------------------------------------------------------------------------------------------------------------------|
| < 50 years  | < 450 pg/mL  | NT-proBNP values greater than 450 pg/mL are consistent with CHF in adults under 50 years of age in the absence of renal failure.        |
| 50-75 years | < 900 pg/mL  | NT-proBNP values greater than 900 pg/mL are consistent with CHF in adults between 50 - 75 years of age in the absence of renal failure. |
| > 75 years  | < 1800 pg/mL | NT-proBNP values greater than 1800 pg/mL are consistent with CHF in adults over 75 years of age in the absence of renal failure.        |

### References:

1. Palazzuoli A, Gallotta M, Quatrini I, et al. Natriuretic peptides (BNP and NT-proBNP): measurement and relevance in heart failure. Vasc Health Risk Manag. 2010;6:411-418.
2. Yancy CW, Jessup M, Bozkurt B, et al. 2013 ACCF/AHA guideline for the management of heart failure: a report of the American College of Cardiology Foundation/American Heart Association Task Force on practice guidelines. Circulation. 2013;128:e240-e319. doi:10.1161/CIR.0b013e31829e8776.
3. Januzzi JL, Richards AM, eds. An International Consensus Statement Regarding Amino Terminal Pro-B-Type Natriuretic Peptide Testing: The International NT-proBNP Consensus Panel. Am J Cardiol. 2008; 101(3A).
4. Josephine M Keyzer, Johannes J Hoffmann, Lianne Ringoir, Karin C Nabbe, Jos W Widdershoven, Victor J Pop. Age- And Gender-Specific Brain Natriuretic Peptide (BNP) Reference Ranges in Primary Care; Clin Chem Lab Med, 52 (9), 1341-6 Sep 2014.

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## BIOCHEMISTRY

|                    |      |      |          |
|--------------------|------|------|----------|
| Troponin (hs-cTnT) | 11.2 | ng/L | (< 14.0) |
|--------------------|------|------|----------|

Note: The reference cut-off above is relevant for the exclusion of AMI for patients presenting with Acute Coronary Syndrome (ACS).

## Risk Stratification:

|              |         |
|--------------|---------|
| Risk         | Cut Off |
| Low          | <5      |
| Intermediate | 5 - <10 |
| High         | >=10    |

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|----------------------|--------------------|------------------------|-----------------|
| <b>LIPID STUDIES</b> |                    | <b>SPECIMEN: SERUM</b> |                 |
| Total Chol           | 总胆固醇               | 3.2                    | mmol/L (< 5.2)  |
| Triglyceride         | 三酸甘油酯              | 1.41                   | mmol/L (< 1.70) |
| HDL-C                | 高密度脂蛋白(好)胆固醇       | 1.54                   | mmol/L (> 1.20) |
| LDL-C                | 低密度脂蛋白(坏)胆固醇       | 1.01                   | mmol/L (< 2.60) |
| Non-HDL              | 非高密度脂蛋白胆固醇         | 1.66                   | mmol/L (< 3.40) |
| T Chol/HDL ratio     | 总胆固醇与高密度脂蛋白胆固醇     | 2.1                    |                 |
| LDL Chol/HDL ratio   | 低密度脂蛋白胆固醇/高密度脂蛋白比率 | 0.66                   |                 |
| AIP                  |                    | < 0.01                 |                 |

| Global Risk                                                                     | Target LDL-C Level | Target non-HDL-C Level |
|---------------------------------------------------------------------------------|--------------------|------------------------|
| Low CV Risk                                                                     | <3.0               | <3.8                   |
| Moderate CV Risk                                                                | <2.6               | <3.4                   |
| High CV Risk                                                                    | <=1.8              | <=2.6                  |
| Very High CV Risk                                                               | <=1.4              | <=2.2                  |
| Recurrent CV events within 2 years despite achieving LDL-C target of <1.4mmol/L | <1.0               |                        |

Reference: 2023 Clinical Practice Guidelines for the Management of Dyslipidemia (6th Edition). Kuala Lumpur: Joint Publication of the Ministry of Health Malaysia, Academy of Medicine Malaysia, National Heart Association of Malaysia.

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|---------------------------------------------|------------------------------------|-----------|-------|--------------------------|
| CV risk indices                             | Category (Gender or Risk Category) | CV risk   |       | Target to reduce CV risk |
| Total Cholesterol /HDL-c ratio (Castelli I) | Men                                | >5.0      |       | <4.5                     |
|                                             | Women                              | >4.5      |       | <4.0                     |
| LDL-c/HDL-c Ratio (Castelli II)             | Men                                | >3.5      |       | <3.0                     |
|                                             | Women                              | >3.0      |       | <2.5                     |
| Atherogenic Index in plasma (AIP)           | Low                                | <0.10     |       |                          |
|                                             | Intermediate                       | 0.10-0.24 |       |                          |
|                                             | High                               | >0.24     |       |                          |

## References:

1. Castelli WJ, Garrison RJ, Wilson PWF, Abbot RD, Kalousdian S, Kannel WB. Incidence of coronary heart disease and lipoprotein cholesterol levels. The Framingham Study. JAMA. 1986;256:2835.2838.
2. Dobiasova M, Frohlich J. The plasma parameter log (TG/HDL-C) as an atherogenic index: correlation with lipoprotein particle size and esterification rate in apoB-lipoprotein-depleted plasma (FER(HDL)). Clin Biochem. 2001;34:583.588. doi:10.1016/s0009-9120(01)00263-6

Note: Effective 15/01/2024, changes in Lipids reference cut off.

## REPORT COMPLETED

## Tests Requested:

FULL LIVERFAST TAGON, ALBUMIN/CREATININE RATIO, INSULIN-HOMA IR&BETA CELL,NT-PROBNP, SPECIAL LIPIDS, HS TROPONIN T, HAEMATOLOGY GENERAL, GLUCOSE, SERUM/PLASMA, LIPID STUDIES, MULTIPLE BIOCHEM ANALYSIS, HAEMOGLOBIN A1C (HBA1C), GLUCOSE, SERUM/PLASMA

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